MSC MARRIAGE AND FAMILY THERAPY

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MFT 719

Family Life Cycle and Transitions

15 teaching weeks · 3 practicals · 0 worked cases · 2 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 703, 715

> “Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 719 — Family Life Cycle and Transitions

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	**Family Life Cycle and Transitions**	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate. This course teaches you to locate the symptom in time: at which stage is this family, which transition is it facing, and what must be renegotiated.

How to use this book

The life-cycle framework is a diagnostic instrument, not a moral timetable. Each week applies it to a different stage and a different kind of transition, including the ones that do not fit the classic sequence — migration, polygyny, foster care, sudden death, disability. Culture is not an exception to the model; it is the material from which the timetable is made.

1. Course description

The course covers the Carter–McGoldrick stage model; the distinction between predictable (normative) and unpredictable (non-normative) transitions; horizontal and vertical stressors; the family's need to renegotiate roles, rules and boundaries at each transition; rituals; intergenerational legacies; and the transitions particular to contemporary African and diasporic family life.

2. Learning outcomes

#	Outcome	Assessed
LO1	Place a family within the life-cycle model and identify its transition	Case analysis
LO2	Distinguish normative from non-normative transitions and their clinical implications	Exam
LO3	Analyse horizontal and vertical stressors in a family	Case analysis
LO4	Design a transition plan with a family	Observed session
LO5	Use rituals and marking to consolidate change	Skills test
LO6	Adapt the model for migration, polygyny, fosterage and non-linear trajectories	Essay
LO7	Work with loss and unfinished transitions	Reflective portfolio

3. Assessment

Component	Weight	Due
Life-cycle case analysis (2,000 words)	30%	Week 7
Transition plan observed session (40 min)	30%	Week 11

Component	Weight	Due
Essay on cultural adaptation (1,500 words)	20%	Week 13
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why transitions produce symptoms	Transition timeline
2	The six stages and their tasks	Stage identification
3	Normative and non-normative transitions	Stressor mapping
4	Horizontal and vertical stressors	Genogram + timeline
5	Stage 1–2: leaving home and joining	Case analysis
6	Stage 3: young children and the couple's reorganisation	Case analysis
7	**Case analysis due**; Stage 4: adolescents	Family session practice
8	Stage 5: launching and the empty nest	Couple re-negotiation
9	Stage 6: later life, caregiving and loss	Caregiver family meeting
10	Migration and displacement as family transitions	Migration genogram
11	**Transition plan observed**	Assessed
12	Divorce, remarriage and blended families	Structural adaptation
13	**Essay due**; births, deaths and unfinished transitions	Ritual design
14	Rituals, anniversaries and marking	Ritual design lab
15	Integration: the family in time	Case presentations

Week 1 — Why transitions produce symptoms

The thesis. Symptoms appear at points of transition because the family's existing organisation is no longer adequate to the new demands, and the renegotiation has not yet happened.

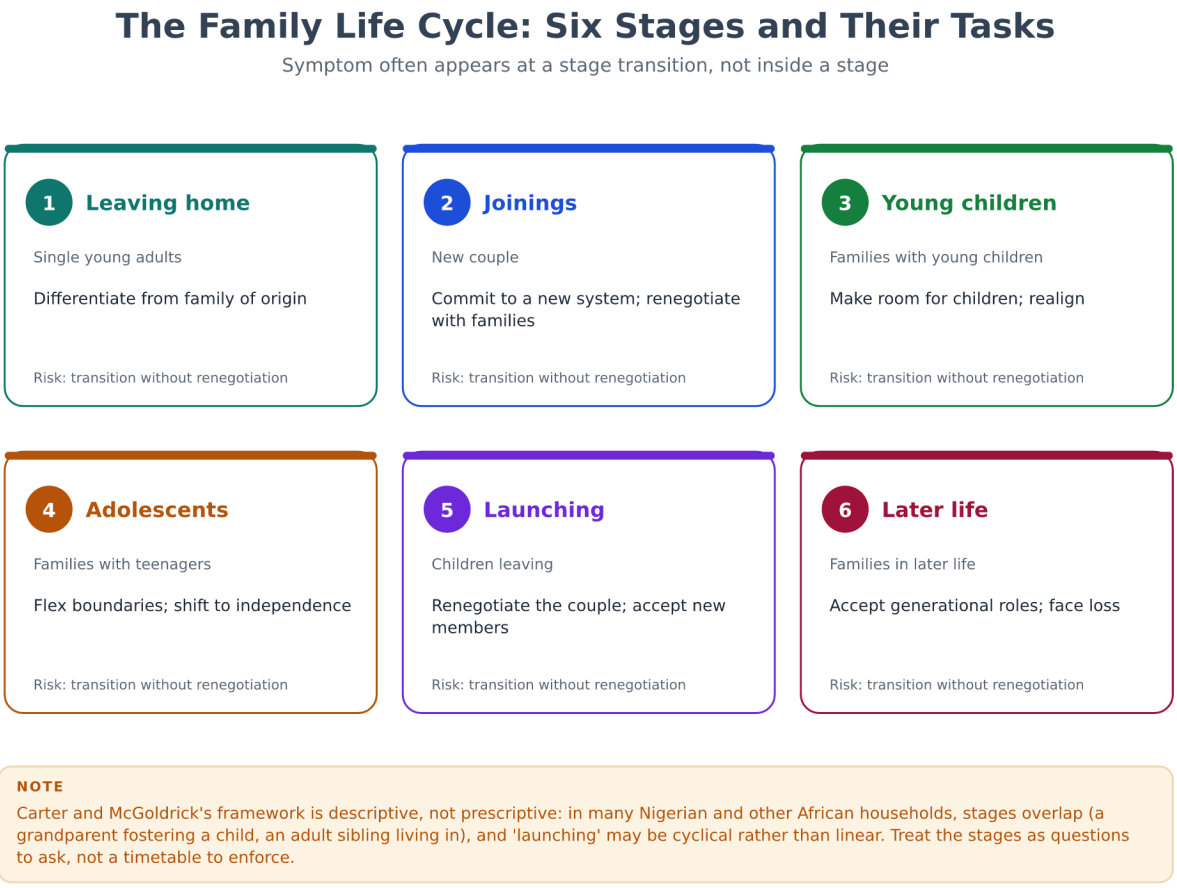


Figure 1.1 — Six stages of the family life cycle and their tasks

Three mechanisms.

1. **Role vacuum.** A role disappears (the working mother, the eldest child at home) and no one has been assigned the new one.
2. **Boundary mismatch.** A boundary that suited the previous stage is now too rigid or too diffuse (parents who cannot let an adolescent have privacy; an adolescent who has become the mother's confidante).
3. **Unfinished business.** The previous transition was never completed (a father who never really left home; a family that never mourned a stillbirth), so the new one cannot be navigated.

Clinical question. "What changed in the last 6–18 months?" almost always produces the precipitant.

> **PRACTICAL**
> Draw a timeline of your family of origin marking moves, births, deaths, marriages, migrations, and job changes. Mark where symptoms or conflicts appeared. Note the pattern.

Week 2 — The six stages

Stage	Emotional task	Second-order change required
1. Leaving home	Accept emotional and financial responsibility	Differentiate from family of origin; develop intimate peer relationships
2. Joining	Commit to a new system	Form a couple; realign relationships with extended families
3. Young children	Accept new members	Adjust the couple system; co-parent; realign with grandparents
4. Adolescents	Increase flexibility	Shift parent–child boundaries; refocus on midlife marriage and careers
5. Launching	Accept many entries and exits	Renegotiate the marriage as a dyad; include in-laws and grandchildren
6. Later life	Accept shifting roles	Deal with loss, disability, and the next generation's centrality

Two warnings. The model is **descriptive of a particular Western, middle-class trajectory**. And the stages overlap: in many families, launching and caregiving occur simultaneously, children return home, and grandparents raise grandchildren.

Week 3 — Normative and non-normative transitions

Normative transitions are expected: marriage, first child, school entry, adolescence, launching, retirement, death of elderly parents.

Non-normative transitions are unexpected or off-time: divorce, a child's disability, a death of a young parent, migration, redundancy, imprisonment, a pandemic, infertility, a child born outside marriage.

Why the distinction matters. Normative transitions have cultural scripts — rituals, advice, role models. Non-normative transitions usually do not, which leaves families improvising under stress, often with shame.

Clinical implication. For non-normative transitions, part of the work is script-making: helping the family construct a way to do something no one they know has done.

> **PRACTICAL**
> Take a non-normative transition (e.g. a child's diagnosis). List the scripts that exist, the scripts that do not, and what the family has had to invent.

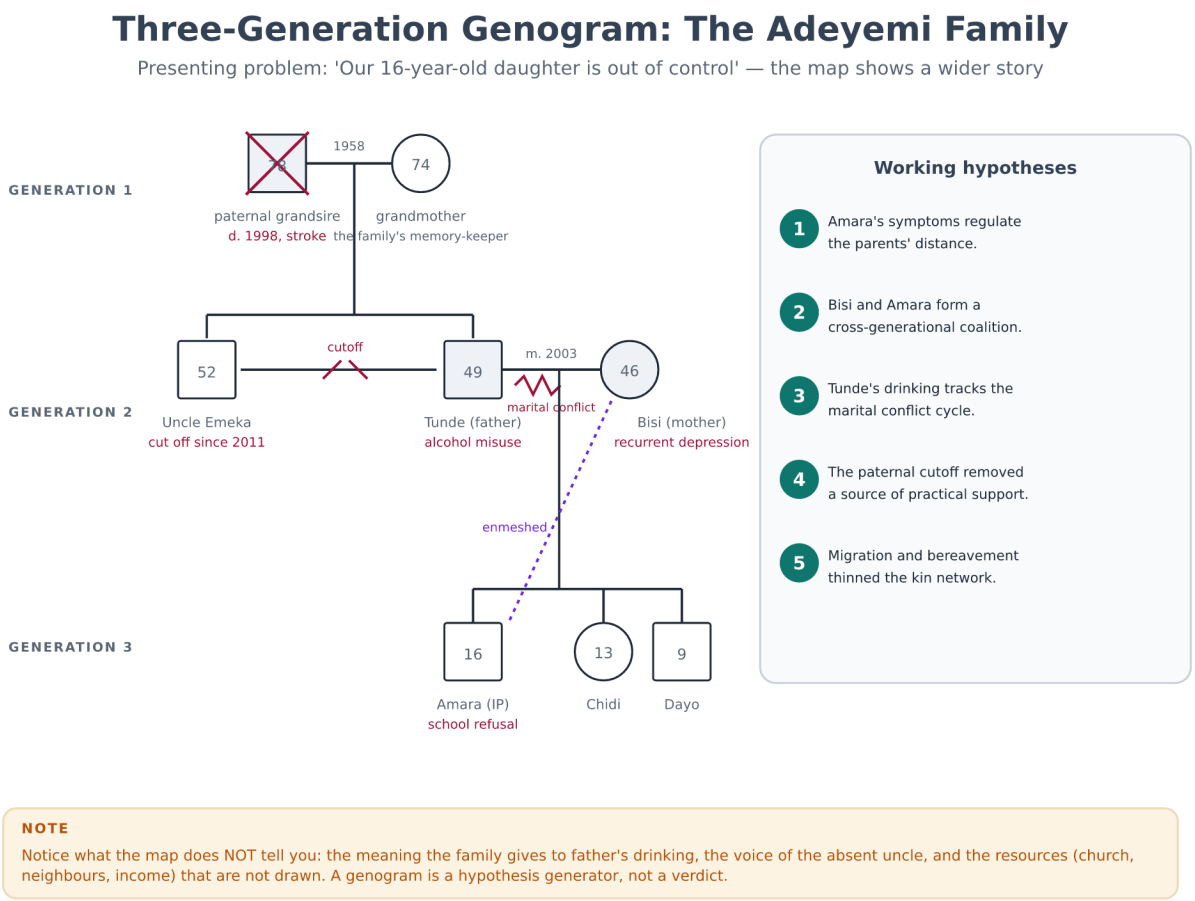
Week 4 — Horizontal and vertical stressors

Horizontal stressors — events moving through time: life-cycle transitions, illness, accidents, migration, poverty, discrimination.

Vertical stressors — legacies handed down: family myths, expectations, secrets, taboos, ethnic and religious beliefs, scripts about gender and success.

The clinical insight. A family can usually manage horizontal stressors or vertical ones; the crisis comes when both arrive at once.

Example: A family facing redundancy (horizontal) in which the father's family has a myth that "a man who cannot provide is worthless" (vertical). The financial problem is manageable; the collision is not.



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 4.1 — A genogram as a map of vertical stressors

Week 5 — Leaving home and joining

Leaving home. The task is differentiation, not departure. A young adult may live at home and be differentiated, or live abroad and be fused. The clinical markers: can they disagree with a parent without catastrophe? Can they ask for help without regression?

Joining. The couple must build a "we" while remaining connected to two families of origin. The most common failure is a boundary problem: a spouse who remains more loyal to a parent than to the partner.

Assessment questions.

- "When you disagree with your mother, what happens?"
- "Who did you consult first about this decision?"
- "How often do you see each family? Who decided that?"
- "Whose opinion matters most in this marriage?"

Week 6 — Young children

The reorganisation. Time, sleep, money, division of labour, sexuality, identity, and the relationship with grandparents all change at once. Research consistently shows a dip in relationship satisfaction after the first child, and that the quality of the co-parenting alliance predicts child outcomes more strongly than conflict.

The clinical presentations.

1. **The couple disappears into parents.** No couple time, no sexuality, no conversation that is not about the child.
2. **The grandmother becomes a third parent.** A cross-generational coalition forms; the father becomes peripheral.
3. **The child becomes the regulator.** A toddler's behaviour organises the household's emotional life.

Intervention. Protect the couple subsystem (time, privacy, an agreement about the division of labour); clarify the parental subsystem (a united front, agreed rules); and negotiate explicitly with grandparents about their role.

Week 7 — Adolescents

The family task is to increase flexibility: more autonomy with more accountability; less control and more influence; boundaries that permit privacy without permitting secrecy.

Two failure patterns.

Pattern	What it looks like	Consequence
Escalation	Parents tighten as the adolescent pushes	Conflict, running away, secrecy, risk
Abdication	Parents withdraw to avoid conflict	Unsupervised risk, no structure, anxiety

The middle path is negotiation with limits: clear non-negotiables (safety), negotiable domains (time, phone, clothes), and consequences agreed in advance.

Also at this stage: the parents' own midlife renegotiation, which often appears as an "adolescent problem" in the referral.

Week 8 — Launching and the empty nest

The task is to renegotiate the marriage as a dyad, and to accept new members and exits (partners, grandchildren, returns home).

The common presentations.

- **The couple who have nothing left to say.** They organised everything around the children; now there is silence.
- **The parent who cannot launch.** A mother who sabotages the child's independence because her role depends on being needed.
- **The child who will not leave.** Economic dependency, anxiety, or a parent's covert invitation.

Intervention. Rebuild the couple relationship deliberately; address the parent's identity and loss; and set a graduated plan for independence that neither humiliates the young adult nor abandons them.

Week 9 — Later life

The tasks. Accept generational role changes; deal with loss (parents, siblings, spouse, friends); support the next generation without taking over; prepare for disability and death; and make meaning of a life.

Clinical presentations. Caregiving disputes among siblings; a widowed parent's new relationship; the "responsible child" who carries everything; inheritance conflict; a grandparent raising grandchildren.

Intervention. Make the loyalty ledger explicit (who has given what, and what is owed); negotiate caregiving tasks concretely; and address the unspoken grief that usually sits under the conflict.

The dual-process frame for grief (Figure 22 in MFT 712) helps here: restoration-oriented activity (tasks, roles, legal arrangements) is not avoidance; it is part of grieving.

Week 10 — Migration and displacement

Migration is a family transition with all the features of a life-cycle stage and none of the rituals.

Phase	Family task	Common difficulty
Decision	Who goes, who stays, who pays	Coercion, debt, secrecy
Separation	Maintain attachment across distance	Children left with relatives; marital drift
Arrival	Establish a household	Status loss, housing, language

Phase	Family task	Common difficulty
Reunification	Re-form as a family	Children who do not know the parent; new authority conflicts
Settlement	Integrate without dissolving identity	Second-generation conflict; "you are not like us"

The "astronaut" and "parachute" patterns. A parent working abroad while the family remains; a child sent abroad for education. Both produce high achievement and high loneliness, and often a "satellite child" who is expected to justify the family's sacrifice.

Clinical questions. Who decided? Who was left? What was lost (status, language, community, identity)? What does the family owe the ones who stayed?

Week 11 — Observed transition plan session

Forty minutes with a role-played family at a transition (launching, caregiving, migration reunification). Assessed on: accurate location of the transition; renegotiation of roles and boundaries; inclusion of all members; and a concrete, reviewable plan.

Week 12 — Divorce, remarriage and blended families

Divorce as a transition, not an ending. The spousal relationship ends; the parental relationship continues in a different form. The family's task is to reorganise across two households while protecting the children from conflict.

The stages. Decision → telling the children → practical reorganisation → legal process → emotional adjustment → new equilibrium (often 2–4 years).

Remarriage and stepfamilies. A new structure with different tasks:

- Build the couple bond before the stepparent assumes authority.
- Negotiate household rules across two systems.
- Allow children to love both households without disloyalty.
- Expect 4–7 years to integrate, not months.
- Address the "insider/outsider" split explicitly.

Clinical error. Treating the stepfamily as a nuclear family that started late, and applying a nuclear family timetable.

Week 13 — Births, deaths and unfinished transitions

Unfinished transitions are those that were never marked, mourned or renegotiated: a stillbirth nobody mentions, a migration that was supposed to be temporary, a divorce that was never explained to the children, a death the family does not discuss.

Signs. Anniversary reactions, a name that cannot be spoken, a child named after the dead, a room kept unchanged, a recurring argument with no present cause.

Intervention. Name the unfinished transition gently; help the family construct a way to mark it; and allow the grief that the avoidance was managing.

Week 14 — Rituals

Rituals consolidate transitions. They mark the boundary between the old state and the new, assign new roles, and connect the family to its wider culture.

Types. Cultural and religious rites (naming, marriage, burial); family rituals (Sunday meals, holidays, anniversaries); and therapeutic rituals (a letter, a ceremony, a shared object, a family meeting).

Designing a therapeutic ritual.

1. **Purpose.** What transition needs marking?
2. **Participants.** Who must be present, and who must be absent?
3. **Symbol.** What object, place, word or action carries the meaning?
4. **Structure.** Beginning, middle, end.
5. **Follow-up.** How will the change be maintained?

Caution. Respect the family's religious and cultural boundaries; do not invent rites that displace their own; and never use a ritual to bypass a necessary conversation.

> **PRACTICAL**
> Design a ritual for a family that has never marked a bereavement. Present it in five minutes.

Week 15 — Integration: the family in time

The life-cycle lens gives you three questions that belong in every assessment:

1. **Where is this family in time?** (Stage, and the transition in progress.)
2. **What must be renegotiated?** (Roles, boundaries, loyalties, rules.)
3. **What has not been finished?** (Losses, transitions, legacies.)

Ask them in the first session and you will rarely be short of a focus.

Glossary

Horizontal stressors — events and transitions moving through time. **Vertical stressors** — legacies, myths and expectations transmitted across generations. **Normative transition** — an expected, culturally scripted life event. **Non-normative transition** — an unexpected or off-time event. **Second-order change** — change in the system's organisation, not only in its behaviour. **Unfinished transition** — a transition that was never marked or renegotiated. **Satellite child** — a child sent abroad or away for education, carrying the family's hopes.

Self-assessment

Multiple choice

1. Symptoms most often appear: (a) randomly (b) at transitions requiring renegotiation (c) only in adolescence (d) only after trauma

2. Vertical stressors are: (a) events in time (b) legacies and myths across generations (c) financial problems (d) illnesses

3. The task of "leaving home" is: (a) departure (b) differentiation while remaining connected (c) financial independence only (d) marriage

4. The dip in couple satisfaction after a first child is: (a) rare (b) a well-documented common pattern (c) caused by the child (d) unavoidable

5. The two adolescent failure patterns are: (a) escalation and abdication (b) fusion and cutoff (c) pursuit and withdrawal (d) enmeshment and disengagement

6. Blended family integration typically takes: (a) months (b) 4–7 years (c) weeks (d) one year

7. A "satellite child" is: (a) a child in orbit (b) a child sent away for education carrying family hopes (c) an only child (d) a foster child

8. Unfinished transitions often present as: (a) clear grief (b) anniversary reactions and unspoken names (c) financial problems (d) delinquency

9. Therapeutic rituals should: (a) replace the family's own rites (b) respect and complement them (c) be religious (d) be avoided

10. The life-cycle model is best used as: (a) a timetable to enforce (b) a descriptive diagnostic lens (c) a legal standard (d) a diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Distinguish normative from non-normative transitions with two examples each.
2. Explain horizontal and vertical stressors with a case.
3. What must be renegotiated when a first child is born?
4. Name four features of migration as a family transition.
5. Give five elements of a therapeutic ritual.

Essay (choose one, 2,000 words)

1. "The life cycle is a Western timetable." Critically evaluate and adapt the model for Nigerian families.

2. Analyse a family's presenting problem as a transition problem, and show what changed in your treatment plan.
3. Discuss the role of rituals in consolidating therapeutic change.

Further reading

- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- McGoldrick, M., Garcia-Preto, N., & Carter, B. (2016). The Multicultural Family Life Cycle. In The Expanding Family Life Cycle (5th ed.).
- Falicov, C. J. (2007). Working with transnational immigrants. Family Process, 46(2), 157–171.
- Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford.

Next: MFT 720 — Supervision and Consultation in Therapy